

ONTARIO
SUPERIOR COURT OF JUSTICE

BETWEEN:

JOCELYNE CRANLEY

Plaintiff

– and –

ANTONIO BAGGIERI and ANTONIO
NIELI

Defendants

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) M. Shannon, for the Plaintiff/Responding
) Party

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) M. O'Brien, for the Defendant/Moving Party
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) **HEARD:** January 6, 2015

RULING ON MOTION

VALLEE J.

- [1] The defendants bring this motion for summary judgment for an order dismissing the action on the grounds that the plaintiff's claim was issued outside of the limitation period. The plaintiff, Ms. Cranley, was injured in a motor vehicle accident on November 25, 2003. She was a 14 year old pedestrian. As result of the motor vehicle accident, she suffered significant injuries including a closed head injury and a fractured left femur.
- [2] The fracture healed well; however, Ms. Cranley had problems with depression and anxiety which persisted. She developed a significant problem with her left knee.
- [3] Ms. Cranley turned 18 on May 24, 2007. She states that she could not have known that her mental health issues and her knee problem were related to the accident within two years of her eighteenth birthday. Her claim was issued on April 27, 2011. The defendants state that there was a body of evidence prior to April 28, 2009 that indicated Ms. Cranley had injuries that exceeded the threshold.

The Issue

Is discoverability a genuine issue for trial?

Chronology

- [4] Shortly after the accident, Ms. Cranley underwent surgery in which two flexible rods were inserted into her leg to repair the fracture. She also sustained a skull base fracture in the right frontal bone. She had memory issues. In fact she does not recall the accident. She noticed taste and smell changes. She had difficulty with language formulation as a result of the head injury.
- [5] On December 2, 2003, she was discharged from hospital. Most of Ms. Cranley's complaints were resolved within weeks or months of the accident.
- [6] In September, 2004, Ms. Cranley started grade 10. She was more or less back to normal. Her only continuing problem was a sense of feeling easily distracted and lacking focus. This impacted her school work.
- [7] In May, 2005, Ms. Cranley was seen by speech language pathologist and an occupational therapist. The speech language pathologist concluded that there were "no difficulties that stemmed from her present injuries" but recommended that she continue to be monitored. Ms. Cranley was also experiencing depression.
- [8] On June 16, 2005, Ms. Cranley underwent neuropsychological testing because her school grades were slipping. Dr. Ladowsky-Brooks concluded that Ms. Cranley had normal functioning in most areas but with "a few mild weakness...consistent with mild residual cognitive dysfunction." She also demonstrated some emotional difficulties relating to depression and anxiety. Dr. Ladowsky-Brooks concluded, "although the academic problems reported are consistent with brain injury, anxiety may also be a contributing factor to the scholastic problems." No opinion was offered as to whether the emotional difficulties were caused by the head injury.
- [9] In September, 2005, Ms. Cranley's mother began submitting to her accident benefits carrier expenses for Ms. Cranley's tutoring from September 2005 to November 2007 as well as for anger management sessions to May, 2007. Ms. Cranley did not know that her mother was submitting these expenses.
- [10] Ms. Cranley had a difficult relationship with her father. She attributed her depression and anxiety to the conflict with her father. She did not undergo any further assessments for the remainder of high school. She graduated in 2006 from grade 12.
- [11] By May 24, 2007, Ms. Cranley's eighteenth birthday, she felt she had recovered well from the accident. She had no ongoing physical problems and did not think her sense of distractibility was a serious problem. In June, 2007, Ms. Cranley enrolled in the interior design program at Sheridan College.

- [12] In January, 2008, Ms. Cranley began to experience pain and swelling in her left knee. She was admitted to hospital and was diagnosed with possible septic arthritis. At that time, she underwent arthroscopic surgery for her knee. While she was in hospital, she was seen by a rheumatologist, Dr. Lewtas who expressed uncertainty as to whether the swelling was caused by early inflammatory arthritis, an isolated arthritis or synovitis related to the 2003 accident and the hardware in her left leg. At that time, nobody told Ms. Cranley that her knee problems might be connected to her 2003 accident. She was discharged with a diagnosis of mono-arthritis.
- [13] On March 5, 2008, an MRI was performed on Ms. Cranley's left knee. The conclusion was "no evidence of internal derangement. Soft tissue edema."
- [14] In May, 2008, Ms. Cranley was having trouble keeping up with school. She left college and moved back in with her parents. The conflict with her father continued. Her depression worsened. She made two suicide attempts and was admitted to hospital after the second attempt for a self-inflicted drug overdose. She was seen by Dr. McNestry, a psychiatrist, who noted her situation with her father and provided a diagnosis of "adjustment reaction." His impression was of a "resolving situational crisis." Nobody at the hospital told Ms. Cranley that her suicide attempts or underlying feelings of depression might be caused by the 2003 head injury. She believed her main problem was with her father.
- [15] In August, 2009, Ms. Cranley's knee flared up again. On November 9, 2009, Dr. Yen, a rheumatologist wrote, "given her previous history of left femur fracture, I am wondering about the possibility of an orthopaedic problem." He referred her to Dr. McKenzie, an orthopaedic surgeon.
- [16] On December 9, 2008, Dr. McKenzie told Ms. Cranley that her knee problems might be related to the protrusion of pins that had been placed in her leg following the accident. She decided to have the pin removed. This was done on February 2, 2010.
- [17] By May, 2010, Ms. Cranley's problems with depression worsened. She made two further suicide attempts. After the first attempt, she was admitted to hospital and discharged the same day with a recommendation to pursue counselling. After the second attempt on May 26, 2010, Ms. Cranley was admitted to hospital where she was seen by a number of doctors including two psychiatrists. Dr. Katz concluded that she was suffering from a major depressive episode and admitted her to the mental health unit. Dr. Parasmorthy stated that Ms. Cranley was not clinically depressed. She exhibited borderline personality traits of affective instability, impulsivity and intense anger. He recommended a CT scan of her brain to "rule out underlying organic pathology given her history of head injury." It was only after this that Ms. Cranley realized that her depression might be caused by her head injury.
- [18] Ms. Cranley continued to struggle with depression. She made another suicide attempt in February, 2012. She was seen a neuropsychologist, Dr. Brandys. Dr. Brandys stated that

Ms. Cranley's responses to testing "were consistent with an overall 'severe' level of depression...and mild anxiety."

- [19] Ms. Cranley continued to have pain and swelling problems with her left knee. She underwent two more surgeries. In a medical/legal report dated June 2013, Dr. Whalen stated that she has an arthritic knee and will require future interventions. He further stated that he was certain Ms. Cranley will need a knee replacement at a young age.
- [20] In early 2011, Ms. Cranley first consulted with counsel because her mother was concerned that the accident benefits carrier would not pay the continuing medical expenses. On April 27, 2011, Ms. Cranley commenced this action.

The Test

- [21] In a motion summary judgment, the court must determine whether there is a genuine issue requiring a trial.
- [22] The *Insurance Act* and the *Limitations Act* apply to this matter. The *Insurance Act* s. 267.5(5) sets out the test for the threshold. Ontario Regulation 461/96 sets out the criteria for impairment. It states:

4.2(1) A person suffers from a permanent serious impairment of an important physical, mental or psychological function if all of the following criteria are met:

1. The impairment must,
 - i. substantially interfere with the person's ability to continue his or her regular or usual employment, despite reasonable efforts to accommodate the person's impairment and the person's reasonable efforts to use the accommodation to allow the person to continue employment,
 - ii. substantially interfere with the person's ability to continue training for a career in a field in which the person was being trained before the incident, despite reasonable efforts to accommodate the person's impairment and the person's reasonable efforts to use the accommodation to allow the person to continue his or her career training, or
 - iii. substantially interfere with most of the usual activities of daily living, considering the person's age.
2. For the function that is impaired to be an important function of the impaired person, the function must,

- i. be necessary to perform the activities that are essential tasks of the person's regular or usual employment, taking into account reasonable efforts to accommodate the person's impairment and the person's reasonable efforts to use the accommodation to allow the person to continue employment,
- ii. be necessary to perform the activities that are essential tasks of the person's training for a career in a field in which the person was being trained before the accident, taking into account reasonable efforts to accommodate the person's impairment and the person's reasonable efforts to use the accommodation to allow the person to continue his or her career training,
- iii. be necessary for the person to provide for his or her own care or well-being, or
- iv. be important to the usual activities of daily living, considering the person's age.

3. For the impairment to be permanent, the impairment must,

- i. have been continuous since the accident and must, based on medical evidence and subject to the person reasonably participating in the recommended treatment of the impairment, be expected not to substantially improve,
- ii. continue to meet the criteria in paragraph 1, and
- iii. be of a nature that is expected to continue without substantial improvement when sustained by persons in similar circumstances.

The amount of the prescribed deductible is \$30,000.

[23] The limitation period begins to run when the plaintiff should have known that she meets the verbal threshold and that her claim exceeds the \$30,000 deductible.

[24] The *Limitations Act* also applies to this matter. The basic limitation period is two years after the claim was discovered. It states:

5.(1) A claim is discovered on the earlier of,

- (a) the day on which the person with the claim first knew,

- (i) that the injury, loss or damage had occurred,
 - (ii) that the injury, loss or damage was caused by or contributed to by an act or omission,
 - (iii) that the act or omission was that of the person against whom the claim is made, and
 - (iv) that, having regard to the nature of the injury, loss or damage, a proceeding would be an appropriate means to seek to remedy it; and
- (b) the day on which a reasonable person with the abilities and in the circumstances of the person with the claim first ought to have known of the matters referred to in clause (a).

Presumption

(2) A person with a claim shall be presumed to have known of the matters referred to in clause (1) (a) on the day the act or omission on which the claim is based took place, unless the contrary is proved.

The Defendants' Position

Summary Judgment

[25] The defendants state that there is no genuine issue requiring a trial. The action is largely document based so it is an appropriate case for summary judgment. Medical evidence was available to the plaintiff within two years after she turned 18 to show that she had injuries that exceeded the threshold. The defendants states that the court should draw inferences from the evidence. The plaintiff knew or ought to have known she had an injury that would exceed the threshold within the limitation period. The defendants state that they carried out a cross-examination of the plaintiff. She relied on her parents considerably. Her mother was dealing with an accident benefits carrier. On the advice of her mother, Ms. Cranley saw a lawyer. Ms. Cranley's mother also had information that would lead her to believe that a limitation period was running.

[26] In *Hyrniak v. Mauldin*, 2014 SCC 7, the court stated in para. 49,

There will be no genuine issue requiring a trial when the judge is able to reach a fair and just determination on the merits of the motion for summary judgment. This will be the case when the

process (1) allows the judge to make the necessary findings of fact, (2) allows the judge to apply the law to the facts, and (3) is a proportionate, more expeditious and less expensive means to achieve a just result.

- [27] The defendants state that a trial with respect to the limitation period would not be proportionate in these circumstances.

Limitations Act

- [28] A limitation period does not run against a minor. On May 4, 2007, Ms. Cranley turned 18. The defendants state that on May 4, 2009, two years after her eighteenth birthday, the limitation period expired.
- [29] The defendants rely on *Peixeiro v. Haberman*, 1997, CanLII 325 (SCC). Mr. Peixeiro was in accident on October 11, 1990. He was told he had soft tissue injuries and was off work for 13 months. He was involved in a second accident on January 7, 1992 in which he sustained more soft tissue injuries. After this accident, he was off work for five months. In total, he was off work until August of 1992. In June 1992, an x-ray showed a disc protrusion. He commenced an action on December 17, 1993.
- [30] After the first accident, Mr. Peixeiro's injuries would not have met the threshold. The court found that the existence of a cause of action was not reasonably discoverable until Mr. Peixeiro learned that he had a herniated disc. Accordingly, his action was not statute barred. The defendants point out that the court also stated that,

At common law, ignorance of or mistake as to the extent of damages does not delay time under a limitation period. The authorities are clear that the exact extent of the loss of the plaintiff need not be known for the cause of action to accrue. Once the plaintiff knows that some damage has occurred and has identified the tortfeasor...the cause of action as accrued. Neither the extent of damage nor the type of damage need be known. To hold otherwise would inject too much uncertainty into cases where the full scope of the damages may not be ascertained for an extended time beyond the general limitation period. (see para. 17)

- [31] The defendants state that Ms. Cranley had identifiable injuries right after the accident. She had a skull fracture and a left leg fracture.

Due Diligence

- [32] The defendants state that injured persons must use due diligence to determine whether injuries and losses would meet the threshold as prescribed by the *Insurance Act*. The defendants rely on *Soper v. Southcott*, 1998 CanLII 5359 (ONCA). This was a medical malpractice case in which the plaintiff had an infection in her knee two years after

surgery. She subsequently consulted counsel. Medical records were requested and sent to a third party doctor for an opinion. After receiving that report, the plaintiff issued a claim. On page 12, the court stated,

Limitation periods are not enacted to be ignored. The plaintiff is required to act with due diligence in acquiring facts in order to be fully apprised of the material facts upon which a negligence or malpractice claim can be based. This includes acting with diligence in requesting and receiving a medical opinion, if required, so as to not to delay the commencement of the limitation period. In some cases, a medical opinion will be necessary to know whether to institute an action. In other cases, it will be possible to know material facts without a medical opinion, and the medical opinion itself will simply be required as evidence in the litigation. In the latter instances, the time of receipt of the medical opinion is immaterial to the commencement of the running a limitation period.

The defendants state that Ms. Cranley did not act with due diligence in acquiring the facts. She did not need medical reports. The issues that she experienced during high school were sufficient for her to have known that she had a claim.

- [33] The defendants also rely on *Ioannidis v. Hockings*, 1998 CanLII 14322 (Ont. Ct. Gen. Div.). In this case, the court set out one of the main tests for due diligence. It stated,

In practical terms, the question is not whether the plaintiff believes that her injury meets the criteria but whether there is a sufficient body of evidence available to be placed before a judge that, in counsel's opinion, has a reasonable chance of persuading a judge, on the balance of probabilities, that the injury qualifies. When such a body of material has been accumulated, then and only then should the limitation period begin to run. (see p. 11)

- [34] The defendants emphasize that the test is not whether the plaintiff believed her injuries would meet the threshold. It is important to consider the body of evidence from the date of the accident until April 28, 2009, two years before the claim was issued.

- [35] The defendants state that Ms. Cranley's mother was submitting claims to the accident benefits carrier. Accordingly, her mother knew that she was continuing to have problems. Ms. Cranley had five anger management sessions after she turned 18. Ms. Cranley reviewed Dr. Parasmorthy's discharge summary dated June 1, 2010 that states that her depression may have caused by the head injury. Dr. Lewtas' note dated January 13, 2014 states that Ms. Cranley had synovitis related to previous trauma. When Ms. Cranley was 19, she was told that she may have some type of arthritis. The defendants state that Ms. Cranley ought to have made some inquiries about this. It is unusual at 19 to have arthritis. It was not a coincidence that Ms. Cranley fractured her left femur and then

went on to have trouble with her left knee. She should have connected the two. She should have known when she was 19 that she had injuries would meet the threshold. This court should draw the inference that sometime prior to April 28, 2009, Ms. Cranley's injuries met or satisfied the threshold. There was sufficient body of evidence to indicate this. She was required to act with due diligence.

- [36] The defendants also rely on *Smith v. Toronto*, [2005] O.J. No. 1543 (Div. Ct.). Specifically, in para. 24 the court stated,

If our interpretation of the law were to give effect to the position of the respondent, it would effectively give her and other injured persons like her the right to extend any applicable limitation period repeatedly from time to time, without any apparent limit, as she gained greater knowledge of the increasing severity of their [her] injury and its consequences. For obvious reasons, such an interpretation would negate the purpose of limitation periods and cause unwarranted confusion to those affected.

- [37] Ms. Cranley's position is that her injuries improved after the accident. Her case is unique due to the intermittent complaints. The defendants state that there is body of evidence which shows that by the time she was 18, Ms. Cranley should have known that she had a claim.

- [38] The defendants also rely on *Yelda v. Vu*, 2013 ONSC 4973 (Ont. Sup. Ct.). In this matter, the plaintiff's accident occurred on July 19, 2002. She had ongoing, increasing pain that began after the accident and continued. Her pain would spike and become intolerable. On these occasions, she went to the hospital. There was no evidence that she did anything to investigate the pain before November 2009, even though her condition worsened as time went on. In 2009, she had other investigations. Only after this time did she discover that she had threshold injuries. The defendants state that these facts are very similar to the instant case. Ms. Cranley states that she did regularly attend the hospital and her doctor when she had issues. This is similar to what Ms. Yelda did. Like Ms. Yelda, Ms. Cranley did nothing else to investigate the source of her issues. She was reactive rather than proactive. Her injuries were continuous since the accident. Some of them worsened. She had objective orthopaedic injuries.

- [39] In *Yelda*, at para. 34, the court stated,

I am of the view if any reasonable person involved in an accident such as this who had ongoing and increasing pain from the date of the accident would conclude eventually she may well have a serious problem which might be permanent. At the very least a reasonable person in the shoes of the plaintiff, assuming her complaints of pain as she describes them are believed, would conclude that she needed to have her medical situation investigated much earlier than she actually did.

[40] In para. 37, the court went on to state

Any one of these facts may not be sufficient for a reasonable person to conclude she did not have a serious permanent problem but the combination of all of them, over a number of years where there was no improvement, is conclusive in my view to conclude that the plaintiff's assertion that she did not suspect she had a serious permanent health problem from the motor vehicle accident is not credible. She had a duty to investigate, and that duty arose long before May 25, 2009, some seven years after the accident.

[41] The defendants contend that from the age of 14 until she was 20, Ms. Cranley had some improvement which was intermittent. This should have indicated to her that she had serious permanent health problems. She had a duty to investigate long before she did. She had objective injuries immediately after the accident occurred.

Analysis

[42] This is an unusual situation. After Ms. Cranley turned 18, she felt that had recovered well from the accident. She had no ongoing physical problems and did not think that her sense of distractibility was serious or that it was related to the injuries that she sustained in the accident.

[43] As noted above, in January of 2008, Ms. Cranley began to experience problems with respect to swelling and pain in her left knee. She had arthroscopic surgery; however, the surgeon, Dr. Haider, discharged her with a diagnosis of mono-arthritis.

[44] Two months later, Ms. Cranley had a MRI of her knee. The related radiology report concluded that there was soft tissue adema.

[45] In 2008, Ms. Cranley's depression worsened. She made two suicide attempts. Dr. McNestry a psychiatrist attributed Mr. Cranley's depression to a resolving situational crisis.

[46] On February 25, 2009, Ms. Cranley again had problems with her left knee. She attended at the hospital, had x-rays that showed no abnormalities. Subsequently, the plaintiff's knee problems subsided.

[47] On May 24, 2009, a limitation period, being two years after the date when Ms. Cranley turned 18, passed.

[48] Ms. Cranley saw an orthopaedic surgeon on December 9, 2009 and appears to have been told for the first time that her knee problems may be caused by a protrusion of the pins that had been placed in her leg after the initial surgery.

- [49] The plaintiff's left femur fractured healed at which point her leg did not bother her. Subsequently, she developed problems with her left knee. The doctors that she saw while at the hospital were unsure as to the cause of the knee problem. Considering this, Ms. Cranley has a legitimate argument that in 2009 when she was 20 years old, she was not able to determine that the cause of her knee problems related to her injuries in the car accident in 2003.
- [50] After 2010, Ms. Cranley's problems with depression worsened. She made several suicide attempts. She was eventually seen by Dr. Parasmorthy who suggested a CT scan "to rule out any organic pathology in lieu of head injury." This appears to be the first time that any doctor suggested to Ms. Cranley that her problems with depression might be caused by the injuries that she sustained in the car accident. Dr. Parasmorthy's discharge summary is dated June 1, 2010, 11 months after the two year limitation date of May 24, 2009. Again, Ms. Cranley has a legitimate argument that she could not have known that her mental health issues were caused by the car accident injuries. Until 2010, none of her doctors appeared to have made this link. Ms. Cranley had significant problems with her father so it was reasonable for her to assume that her depression was caused by difficulties in that relationship.
- [51] Given the above, Ms. Cranley argues that she could not have known that her knee problems might be related to her leg fracture until she saw Dr. McKenzie on December 9, 2009. This was approximately seven months after the two year limitation period had expired. With respect to her emotional difficulties, Ms. Cranley argues that she could not have discovered that these problems were related to the head injury that she sustained in the car accident until June 1, 2010, the date of Dr. Parasmorthy's report. Both of these arguments have merit. Ms. Cranley's claim was issued on April 27, 2011, within two years after December 9, 2009 and June 1, 2010.
- [52] According to the *Insurance Act* a plaintiff must establish that she suffered a "permanent serious impairment of an important physical, mental or psychological function." Ms. Cranley notes that in *Ioannidis*, the question is not whether she believed that her injuries met the threshold criteria but whether there was a sufficient body of evidence available to be placed before a judge that in counsel's opinion had a reasonable chance of persuading a judge on the balance of probabilities that the injury qualifies. Ms. Cranley argues that there was not a sufficient body of medical evidence that might have persuaded a judge that her injuries were serious and permanent prior to April 28, 2009. In *Eberding v. Skrijel*, 263 O.A.C. 130, the Court of Appeal stated:

Clearly it is not the policy of the law or the intent of the limitations provisions to require people to commence actions before they know that they have a substantial chance to succeed in recovering a judgment for damages. These provisions of the *Insurance Act* were enacted as part of a scheme that provides compensation with no fault benefits for injuries that are not considered to be serious or permanent, but allows actions to proceed where injuries are sufficiently significant that a substantial monetary award is likely

to be recovered. Consequently, the test for the discoverability of the existence of such a claim for limitation purposes must be in accordance with the same policy.

- [53] In *Smith v. Blackhall*, 2012 ONSC 4990, the plaintiff was involved in an accident in which she injured her knees. She was off work for many months but did eventually return. She continued to have daily knee pain. She consulted several doctors some of whom stated that her problems might improve if she lost weight. Eleven years after the accident, she was told that she had degenerative arthritis and would require a knee replacement. After she issued a claim, the defendant brought a motion for summary judgment. It was dismissed. Although the plaintiff had suffered ongoing pain for eleven years, she did not believe she had a serious injury until a doctor advised her that she would require a knee replacement. The court held that a full trial was necessary to make findings as to discoverability.

Conclusion

- [54] Based on the record before me, I conclude that there is a genuine issue requiring a trial with respect to when Ms. Cranley knew or ought to have known that she had injuries which would exceed the threshold. A trial is necessary to make findings regarding discoverability and whether Ms. Cranley's injuries do exceed the threshold. Accordingly, the defendants' motion is dismissed.

Costs

- [55] The plaintiff has successfully defended the motion. Despite the requirement set out in Rule 57.01(b), counsel for the plaintiff did not bring a costs outline and was not prepared to make costs submissions at the end of the motion. Accordingly, the plaintiff shall provide written submissions not to exceed three pages exclusive of a costs outline within three weeks of the release date of this decision. The defendants may provide responding costs submissions within the following two weeks. They shall be sent to my assistant in Barrie, Nicole Anderson at Nicole.Anderson@ontario.ca.

VALLEE J.

Released: March 17, 2015